

Novo Nordisk (NVO) — Research Note : is the obesity leader a bargain, or is Lilly simply winning?

How to read this. This note shows my working. The “lenses” (quality, cycle, valuation, inversion) are **analytical frameworks applied by the author — no third party is involved in, or endorses, this research. This is analysis and general opinion, not a recommendation, and I am not stating a buy/sell/hold view.** As of 2026-07-06; later events not incorporated.

The question

Novo leads the branded obesity market but just lost the head-to-head trial to Eli Lilly's tirzepatide, and the stock has fallen to what looks like an ex-growth multiple. **Is the leading franchise with a wounded next-gen pipeline a bargain — or is Lilly simply winning?**

Company snapshot (at a glance)

A one-look profile of the company. All figures from Novo's public filings (AR 2025 / Form 20-F / 2026 interims); prices are market quotes; market cap, EV and multiples are my own calculation, not quoted. As of 2026-07-06.

What it is: a **Danish pharmaceutical company** — one of the two global leaders in **diabetes and obesity treatment**. Almost all revenue comes from one molecule family, **GLP-1 / semaglutide** — sold as **Ozempic** and **Rybelsus** (diabetes) and **Wegovy** (obesity) — plus a large legacy **insulin** business and a small **rare-disease** arm. It is unusually **vertically integrated** (makes its own active ingredient and does its own fill-finish) and is **controlled by a charitable foundation** (Novo Holdings, ~77% of votes on ~28% of the economics), so ordinary shareholders cannot force a sale. Listed as **NOVO-B** (Copenhagen) and **NVO** (NYSE ADR, 1:1); reports in DKK, which is pegged to the euro.

Market snapshot (2026-07-06)	Value	Business mix – FY2025 (DKK 309.1bn)	%
Price NOVO-B / NVO ADR	DKK 324.0 / \$50.43	Diabetes care	67%
52-wk range (NOVO-B)	224.3 – 464.6	Obesity care (+31% CER)	27%
Market cap ≈	€193–197bn	Rare disease	6%
Enterprise value ≈	€205bn	US Operations	56%
Trailing / forward P/E	~14× / ~15–17×	International	44%
EV/EBITDA · div. yield	~10.3× · ~3.5%	Branded-obesity volume share	59.6%

Five-year signature (DKK m / %)	2021	2023	2024	2025
Net sales (5-yr CAGR 21.7%)	140,800	232,261	290,403	309,064
Gross margin	83.2%	84.6%	84.7%	81.0%
ROIC (disclosed)	69.0%	88.5%	63.9%	39.3%
Cash conversion (FCF/NP)	61%	82%	–15%	27.6%

Key products: Ozempic (≈ the biggest single product), Rybelsus, Wegovy (injection + a new oral pill, US launch Jan 2026), legacy insulins, and a pipeline broader than one molecule (CagriSema – filed but missed the head-to-head; amycletin – a strong next-gen candidate, arguably the real long-term answer to Lilly; plus MASH, cardiovascular and rare-disease assets). **The tension in one line:** Novo is the branded-obesity **volume leader (59.6%)** in a market that grew ~104% in 2025 – yet it **lost the only head-to-head to Lilly** and in 2025 **every quality metric turned down together for the first time** as a huge capacity build-out consumed cash. Whether that is a temporary dent or a permanent worsening is the question the rest of this note works through.

The business

Novo is a Danish, R&D-driven pharma leader in **diabetes and obesity**, built on one molecular platform – **GLP-1 / semaglutide** (Ozempic, Rybelsus, Wegovy) – plus legacy insulin and a small rare-disease arm. By FY2025 revenue it is **~67% diabetes, ~27% obesity, ~6% rare disease** – i.e. still mostly a diabetes company, with obesity the growth engine. It is **foundation-controlled**: the Novo Nordisk Foundation (via Novo Holdings) holds ~77% of the votes on ~28% of the economics – a patient, through-cycle owner (the Danish foundation-compounder model that can fund a heavy capex build-out public markets would punish), though one that limits minority-holder leverage and takeover optionality. All figures from Novo's AR 2025, Form 20-F and 2026 interim filings.

The industry & competition

A **secular, fast-growing market in a duopoly**. The global branded GLP-1 obesity market grew ~104% in 2025 (Novo AR); Novo holds a **59.6% branded volume share**, Lilly most of the rest. But on the momentum metrics **Lilly is ahead**: FY2025 revenue ~\$65bn (+45%) vs Novo ~\$47bn (+6–10%), Zepbound +175% vs Wegovy +31%, and **tirzepatide beat semaglutide** in the only head-to-head (REDEFINE 4: 23.0% vs 25.5%). The next battleground is **oral**: Novo's Wegovy pill (a peptide – higher efficacy, harder to scale) vs Lilly's orforglipron (a small molecule – ~12.4% weight loss, but far cheaper and easier to scale). US pricing is a real headwind – the semaglutide franchise was selected for **IRA Medicare price negotiation** (a ~71% cut on the Medicare channel from Jan 2027).

The financials & what's priced in

An elite but **turning** signature: gross margin 84.7%→81.0%, operating margin ~44%→41.3%, and **ROIC ~88.5% (2023) → 39.3% (2025)** as a huge capacity build-out (capex DKK 6.3bn→60.1bn) plus the Catalent acquisition consumed cash – **free cash flow was actually negative in 2024** before recovering. Growth fell from +25–31% to +6% (DKK) / +10% (CER).

What the price implies (reverse-DCF, my calculation, EUR basis at DKK 324): on normalised earnings, today's price embeds only **~0–3% annual growth for a decade** at an 8–10% cost of equity – a *pessimistic* path for a company that compounded ~21%/yr over 2021–25. **But that only holds if capex normalises so free cash flow recovers**; anchor on today's depressed cash flow and the same price implies a big recovery is already required. So “cheap” is conditional, not automatic.

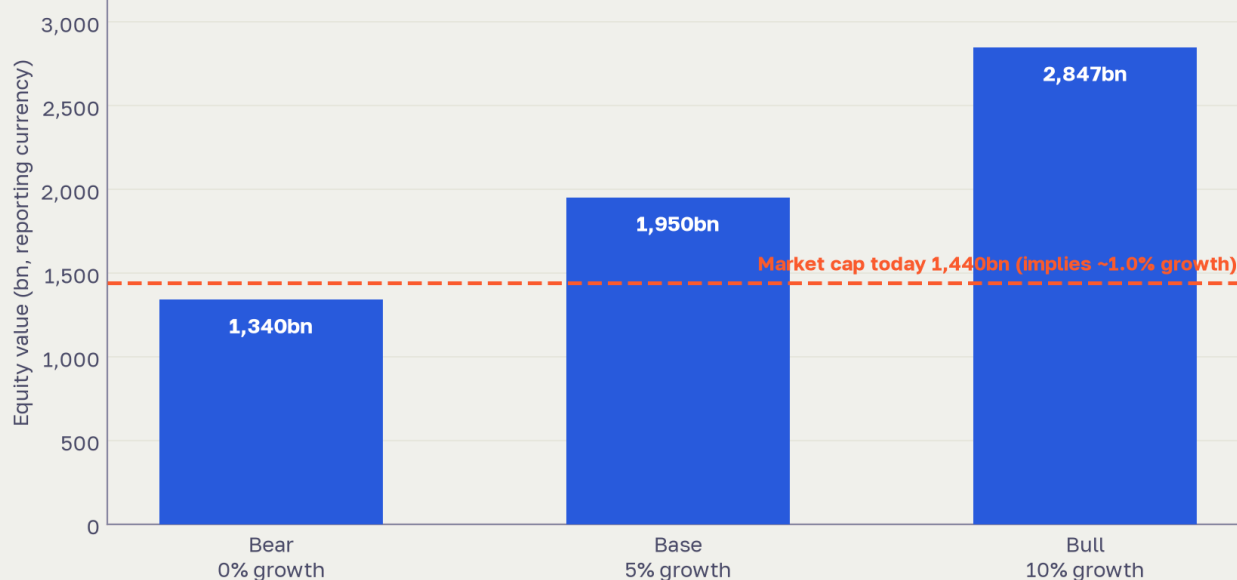
What the price is assuming (valuation & probability)

The market is making a very specific bet on Novo, and it is worth saying out loud.

Work backwards from the share price – a reverse-DCF – and at DKK 324 the market is paying for roughly 1% profit growth a year, for a decade. For a company that grew north of 20% a year through the GLP-1 boom, that is an ex-growth price.

Novo Nordisk — modelled equity value under three growth paths

Equity value if earnings grow at each rate for 10 years (9% discount, 2.5% terminal) — vs today's market cap



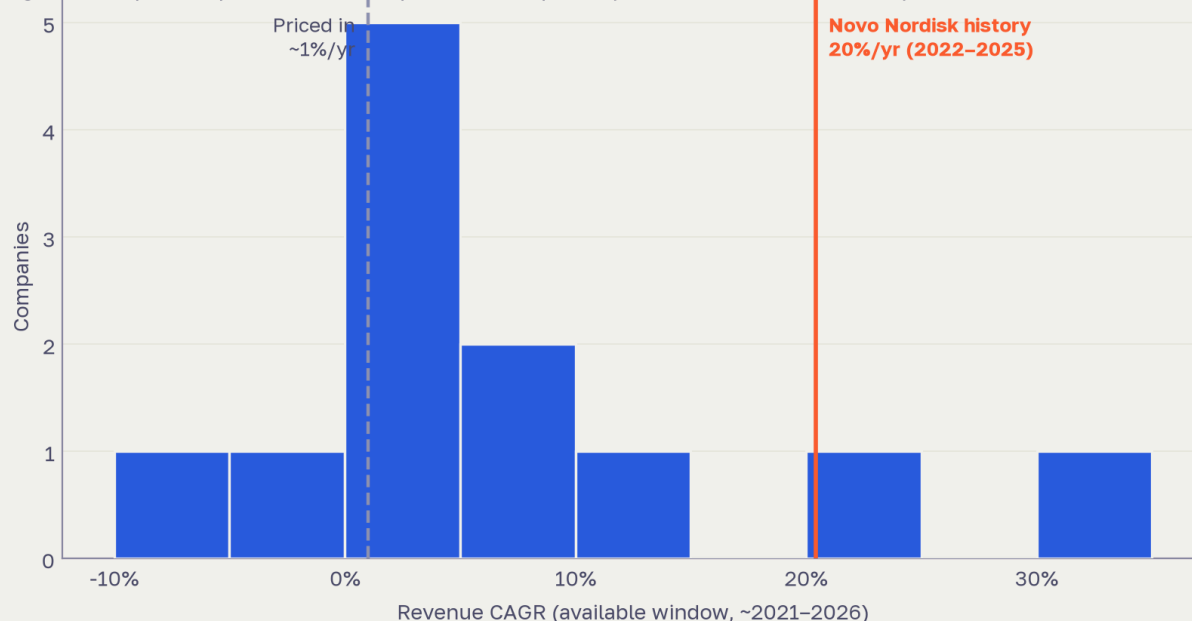
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What the price is paying for: Novo's modelled equity value if earnings grow at 0%, 5% or 10% for a decade — the dashed line is today's market cap, sitting just above the zero-growth bar.

Is 1% a reasonable expectation? To find out, I compared Novo against eleven of its actual peers — Lilly, Roche, Novartis, AstraZeneca, Sanofi, GSK, Merck, Pfizer, AbbVie, Amgen, Bristol Myers. Two things stood out. First, sustained fast growth is rare even in pharma: only a quarter of the group kept revenue growing above 10%, and the median grew about 4% a year. Second, and this is the striking part, Novo's own recent record — about 20% a year — was the *second fastest of the twelve*. Yet the ~1% now baked into its price would rank *tenth* of twelve, below AbbVie and Bristol Myers, ahead of only Roche and Pfizer.

Novo Nordisk vs the pharma basket — revenue CAGR

Revenue CAGR across 12 companies ('pharma' basket, cobalt) vs Novo Nordisk's own record and the growth its price implies. Survivorship-biased sample; ~4y windows — directional only.



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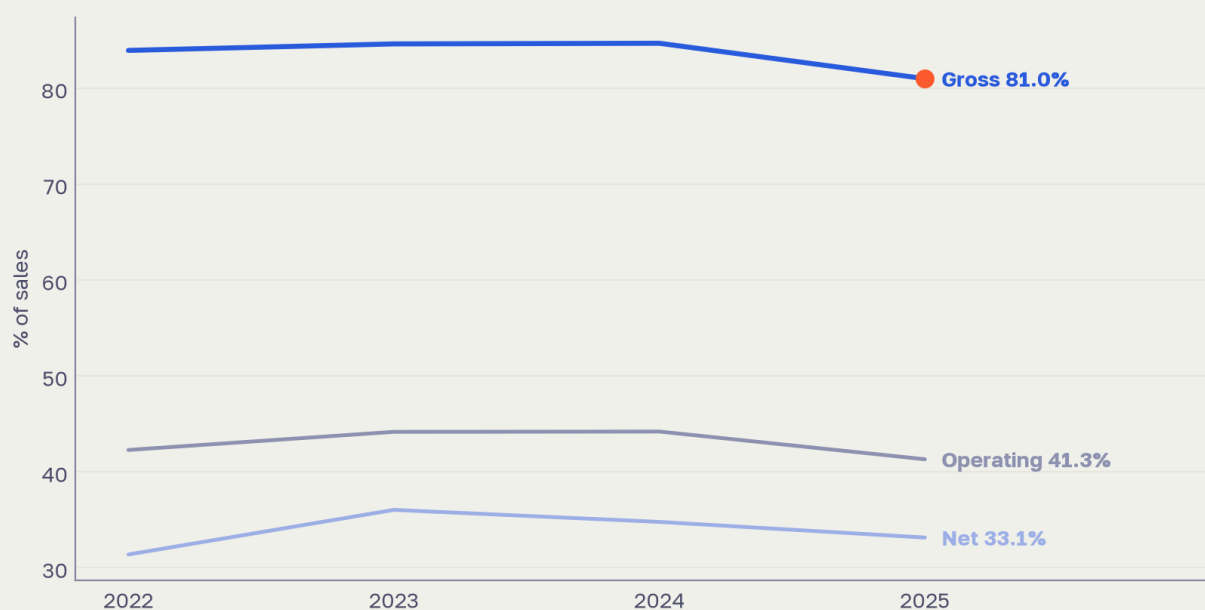
The peer evidence: revenue growth across the twelve-company pharma set. Novo's own record (orange) sits second from the top; the ~1% its price implies (grey dashed) sits near the bottom.

So in the space of roughly a year, the market has repriced the industry's number-two grower as a near-bottom decliner.

That does not make Novo cheap. It makes the whole question a single one: is that repricing right? The bear case is real and specific — Lilly is winning the head-to-head, and a US Medicare price cut lands in 2027. If those bite hard, a decliner's price is fair. If they do not, the market is punishing a proven grower for one difficult stretch.

Novo Nordisk — gross, operating and net margin, 2022–2025

Margins as % of sales, by fiscal year



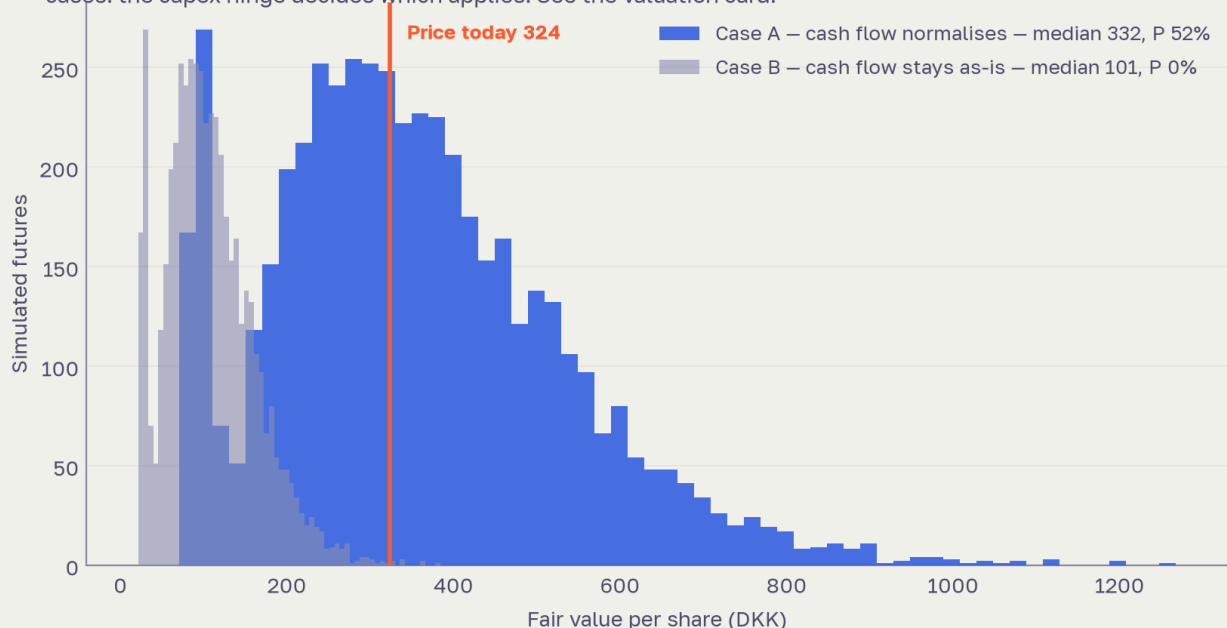
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The difficult stretch, visible: 2025 was the first year gross, operating and net margins turned down together — still world-class levels, but the direction is the story.

Running the valuation five thousand times — growth drawn around the pharma-peer median, a Lilly-plus-Medicare bear included — the answer splits on a single assumption. If Novo's currently depressed free cash flow normalises back toward its profit once the build-out ends, as management guides, fair value clears today's price in about half of plausible futures: a coin flip, not a bargain. If cash flow stays where it is, it clears in none of them. The share price is not really the question. The capex hinge is.

NOVO-B.CO — fair value distribution, 5,000 draws

Growth centred on 'pharma' peer median (n=12) at +4.2%; bear floor -10.0%; break prob 10%. Two cases: the capex hinge decides which applies. See the valuation card.



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The whole argument in one picture: 5,000 simulated futures. If cash flow normalises (blue), today's DKK 324 cuts through the middle — a coin flip. If it stays as-is (grey), the price sits above every future. The gap between the two humps is the capex assumption.

I am not going to tell you which way it breaks, and I am certainly not going to tell you what to do about it. What I can do is show you exactly what the market is assuming, how that compares to Novo's real peers, and where the argument actually turns — so you can weigh it yourself.

Research and education, not investment advice.

The bull case

Expectations are on the floor. A structurally elite franchise (81% gross margin, ~50% dividend payout) leads a market growing ~100%, is *not* a one-molecule company (real pipeline breadth in MASH, cardiovascular, rare disease), just posted the “strongest-ever GLP-1 volume launch” with the oral pill, and has a capex super-cycle that should roll off (per guidance) so cash flow recovers. If the franchise merely *holds*, the priced-in stagnation looks too harsh.

The bear case

A structurally impaired #2 losing a molecular arms race. Lilly is bigger, ~5× faster-growing, wins the head-to-head, and its oral small molecule could commoditise the tier Novo's API-heavy pill depends on. The ROIC collapse may be the market pricing that incremental capital now earns

less; a ~71% Medicare cut lands in 2027; and if the decline is structural, “cheap” is a value trap – on current cash flow the price already needs a recovery that may not come.

Through different investing lenses (perspectives, not a scorecard)

Different well-known investing frameworks – used here as *lenses*, not as endorsements – pull in different directions on this name, which is itself the point: - **Quality lens**: the financial signature is elite by pharma standards, but in 2025 it turned down across the board (margin, ROIC, cash conversion), and the moat – while real – is *narrowing* after the head-to-head loss. - **Cycle / sentiment lens**: the name has swung from euphoria to despair; the price now reflects pessimism, not optimism – a contrarian setup *if* the analysis holds. - **Valuation lens**: undemanding on multiples and on what the price implies (~0–3% growth priced in), but conditional on the cash-flow recovery. - **Inversion / risk lens**: the sharpest risk is that Novo becomes “the leader that loses the head-to-head and doesn’t recover” (the base-rate concern below), with US pricing and pipeline as the mechanisms; the mitigants are its incumbent share, breadth beyond one molecule, and the dividend.

(These are analytical perspectives to weigh – not ratings, not a recommendation.)

Base rates (context)

- **When a pharma #2/leader loses the pivotal head-to-head, does it recover the lead?** Illustrative case (one iconic analogue, not a full dataset): in PD-1, BMS’s Opdivo lost the first-line lung head-to-head to Merck’s Keytruda and **never recovered leadership** – the winner compounded its lead. Directionally: **the loser rarely re-takes the lead**.
- **Do very-high-ROIC companies keep those returns?** Cross-sectional history (Damodaran; Morgan Stanley): high ROIC fades toward the cost of capital, but top-quintile names often **stabilise above it** for years if the moat holds – full return to peak is unlikely, staying above cost of capital is common.
- **Are negotiated price cuts a cliff or a gradual drag?** Historically a **persistent drag**, phased-in and partly offset by volume, rather than an overnight cliff to total revenue.

The questions that decide it

1. Is the moat the **molecule** (losing) or the **franchise** (brand, oral tech, manufacturing, payers)?
2. Is the ROIC / cash-flow collapse **cyclical** (capacity build-out) or **structural** (permanent competitive + capital intensity)?
3. Can the oral pill + pipeline breadth + international volume **offset Lilly and the 2027 price cut** – i.e. does growth re-accelerate?

4. How deep and permanent is the **US net-price reset**? **The swing question is #3** – whether growth actually recovers. It is genuinely uncertain, and it is where the whole case is won or lost.

What I'm watching / what would change the view

- **US Wegovy weekly prescription share vs Zepbound** – the earliest tell on the franchise.
- **2026 landing vs guidance, and whether 2027 sales growth inflects positive.**
- **Whether capex declines as guided and free cash flow recovers toward net income.**
- **Whether the Medicare price cut spreads to commercial channels.**

Why I'm not giving you a verdict — and how to form your own

I keep my own buy/sell/hold decision private, deliberately. Two reasons: my decision depends on *my* circumstances, constraints, and portfolio – which aren't yours – and a public “call” is both less useful and more fraught than showing you how to reason about it. What I *can* do is show the full analysis and the questions I think decide it, and leave the conclusion to you.

To form your own view, sit with these – your honest answers, not mine, decide it for you: 1. **Do you understand this industry well enough** to judge pipeline and regulatory/pricing risk – or is it honestly “too hard” for you? 2. **Cyclical or structural?** Do you think 2025–26 was a temporary dent (capacity build-out, accounting/pricing noise) or a permanent worsening of the competitive position? 3. **Do you already own this inside a broad index fund?** If so, does an *active overweight* clearly earn its place versus simply staying indexed? 4. **What specifically would change your mind** – in either direction – and are you watching for it?

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Analytical lenses. *The analytical "lenses" used in this research (quality, cycle, indexing, inversion, valuation) are frameworks applied by the author. No third party is involved in, or endorses, this research.*

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